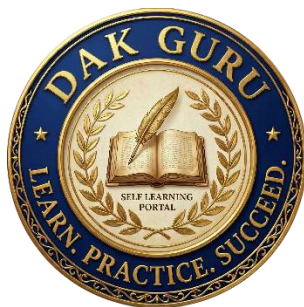




DAK GURU

INDIA'S FIRST SMART SELF-PREPARATION PORTAL

FOR LDCE-IP & PS GROUP B ASPIRANTS

CENTRAL SERVICES (MEDICAL ATTENDANCE) RULES, 1944

Comprehensive Study Material — Revised & Updated Edition

Incorporating amendments, Office Memoranda & notifications up to July 2026

✓ KEY AMENDMENTS INCORPORATED IN THIS EDITION

- Revised AMA consultation / injection fees w.e.f. 20.09.2023 (PG Rs. 350; Graduate Rs. 200)
- Homoeopathy / Indian Systems of Medicine treatment limit — 40 days (4 consultations)
- AMA prescription single-use validity — 2 weeks (revalidation thereafter)
- Revised Ward Entitlement w.e.f. 28.10.2022 (Rs. 36,500 / 50,500 slabs)
- Coronary stents capped per NPPA (DES Rs. 39,186 / BMS Rs. 10,762, w.e.f. 1.4.2026)
- Revised IOL categories & rates per MoHFW OM (S-1413/2005-06/CGHS/R&H)
- Parents / Parents-in-law one-time option — re-clarified w.e.f. 13.05.2026
- RPwD Act, 2016 (21 disabilities) · MTP (Amendment) Act, 2021 · Mental Healthcare Act, 2017



DISCLAIMER

This booklet has been prepared strictly from an examination point of view for LDCE-IP and PS Group B aspirants of the Department of Posts. Every effort has been made to ensure accuracy and to incorporate the most recent amendments, Office Memoranda, notifications, gazette publications and judicial pronouncements affecting the Central Services (Medical Attendance) Rules, 1944 and the Central Government Health Scheme (CGHS) up to July 2026.

*However, this document is intended as a study aid only and does not constitute a statutory or legal reference. Rules, rates and monetary ceilings are periodically revised by the competent authorities. Aspirants and readers are therefore strongly advised to refer to the official Gazette of India, Ministry of Health & Family Welfare circulars (cghs.mohfw.gov.in), DoP&PW / DoP&T orders and the original Government Office Memoranda for authoritative reference, especially in case of any conflict, doubt, or subsequent amendment. **Dak Guru shall not be liable for any decision taken solely on the basis of this material.***

How to use this booklet: Statutory rule text is presented under each **RULE** banner. Administrative clarifications appear under **GOVERNMENT DECISIONS** banners. Wherever a figure or provision has been revised, a colour-coded tag (**UPDATED** / **CORRECTED** / **NEW**) flags the change, and a  **LEGISLATIVE NOTE** cites the governing order and date.  **EXAM TIP** boxes highlight frequently-tested points.

INTRODUCTION & STATUTORY BACKGROUND

The Central Services (Medical Attendance) Rules, 1944 — popularly known as the CS (MA) Rules, 1944 — were notified on 1st January, 1944. These Rules constitute the principal statutory framework governing free medical attendance and treatment to Central Government civil servants and their dependent family members in stations not covered by the Central Government Health Scheme (CGHS).

The Rules were made by the Governor General-in-Council in exercise of the powers conferred by sub-section (2) of Section 241 read with sub-section (3) of Section 313 of the Government of India Act, 1935. After the commencement of the Constitution of India, 1950, these Rules continue in force by virtue of Article 313 (continuance in force of existing laws on conditions of service).

 **EXAM TIP:** CS (MA) Rules and CGHS together form a complementary scheme — CS (MA) Rules apply where CGHS is not in force, while CGHS operates in 80 notified cities (being expanded to 100). For LDCE-IP, focus on (i) AMA fees, (ii) ward entitlement, (iii) family definition, (iv) reimbursement time-limits, (v) treatment outside India, and (vi) special diseases.

RULE 1 — SHORT TITLE AND EXTENT OF APPLICATION

(1) Short Title and Date of Notification

These rules may be called the Central Services (Medical Attendance) Rules, 1944. They were notified on 1st January, 1944.

(2) Categories to whom these Rules APPLY

- All Government servants whose conditions of service are prescribed by Rules made or deemed to be made by the Central Government — while on duty, leave or Foreign Service in India, or under suspension (except those listed in sub-rule (3));
- Government servants on leave preparatory to retirement;
- Government pensioners on their re-employment;
- Defence Accounts Department personnel;
- Civilians paid from Defence Services Estimates;
- Central Government employees on terminal leave;
- Probationers;
- Apprentices in the whole-time service of Government;
- All State Government servants on deputation with the Central Government.

(3) Categories to whom these Rules DO NOT APPLY

- Persons in railway service (covered by Railway Services Medical Attendance Rules);
- Defence Services personnel (covered by ECHS / AFMS);
- Government servants on deputation abroad;
- Non-gazetted personnel stationed in or passing through Calcutta (legacy provision);
- Retired Government officials;
- Non-officials sent on deputation abroad;
- Government servants governed by CGHS while in CGHS-functioning stations;
- Officers of All India Services (covered by AIS (MA) Rules, 1954);
- India-based officers and staff serving in Missions abroad.

**GOVERNMENT DECISIONS / CLARIFICATIONS**

(1) All Government servants and members of their family governed by these Rules, while on tour or on leave visiting any place where CGHS is in force, may avail of CGHS facilities.

(2) Claims in respect of officials admitted before retirement but discharged after superannuation — the expenses shall be reimbursed by the official's own office (parent department).

RULE 2 — DEFINITIONS

(a) **“Authorized Medical Attendant” (AMA)** means a medical officer in the employ of the Central Government, or a private medical practitioner appointed / nominated by the Ministry / Department, for providing medical attendance to its employees.

(b) **“Patient”** means a Government servant to whom these Rules apply and who has fallen ill.

(c) **“Treatment”** means the use of all medical and surgical facilities available at the Government hospital in which the Government servant is treated.

**GOVERNMENT DECISIONS / CLARIFICATIONS**

(1) Appointment of private registered medical practitioners as AMAs is to be done by the Head of the Department concerned. The nomination of an AMA is valid for one year and is renewable.

(2) No treatment shall be obtained from an AMA while he is on regular leave.

(3) Consultation / visiting / injection fees of AMA :

Category of Medical Officer	Consultation Fee	Injection Fee
I. Medical Post Graduate / Specialist	First: Rs. 350/- Subsequent: Rs. 350/-	Intramuscular / Subcutaneous: Rs. 25/-

Category of Medical Officer	Consultation Fee	Injection Fee
		Intravenous: Rs. 50/-
II. Medical Licentiates & Medical Graduates (MBBS or equivalent)	First: Rs. 200/- Subsequent: Rs. 200/-	Intramuscular / Subcutaneous: Rs. 25/- Intravenous: Rs. 50/-

LEGISLATIVE NOTE: The consultation / visiting / injection fees have been comprehensively revised vide MoHFW (Department of Health & Family Welfare) O.M. No. S.11025/18/2023-EHS dated 20.09.2023, superseding the earlier rates fixed vide O.M. dated 31.12.2002 and 17.03.2011. Under the revised structure the fee is the same for the first and every subsequent consultation — **Post Graduate / Specialist Rs. 350/-** and **Graduate (MBBS) Rs. 200/-**. Injection fees are **Rs. 25/- (Intramuscular / Subcutaneous)** and **Rs. 50/- (Intravenous)** for both categories. It is expressly clarified that there are **no separate charges for day and night consultation** any longer. [Ref: MoHFW O.M. No. S.11025/18/2023-EHS dated 20.09.2023]

EXAM TIP: Older study material still quotes the pre-2023 fees (PG Rs. 100/60, Graduate Rs. 70/40, IV injection Rs. 40). For LDCE-IP now remember: **PG/Specialist Rs. 350 (flat), Graduate Rs. 200 (flat), IM/SC injection Rs. 25, IV injection Rs. 50**, and no day/night distinction.

(4) Simultaneous treatment in two or more systems of medicine is permissible only for different ailments.

(5) Reimbursement of charges for clinical tests paid to private practitioners / institutions:

- Up to Rs. 500/- — a certificate from the medical officer in charge of the case in the hospital that such facilities are not available in a Government hospital is required.
- Exceeding Rs. 500/- — prior approval of the Medical Superintendent of the hospital is required.

(6) Ceiling for reimbursement of Special Nursing and Ayah / Attendant:

- Special Nurse: Rs. 150/- per shift of 12 hours.
- Ayah / Attendant: Rs. 75/- per shift of 12 hours.

(7) IVF (In-Vitro Fertilization) Treatment [OM No. Z.15025/5/2011-CGHS III/CGHS(P) dated 22.11.2011]:

- Age of woman undergoing IVF: between 21 and 39 years.
- The woman must be married and living with her husband.
- Permitted only in cases of infertility where the couple has no living issue.
- Reimbursement allowed up to a maximum of 3 (three) fresh cycles.
- Amount: not exceeding Rs. 65,000/- per cycle or actual cost, whichever is lower (inclusive of drugs, disposables and monitoring).

- Prior approval mandatory; permission only on the recommendation of the HOD, Gynaecology & Obstetrics of a Government Medical Institution.

(8) Validity of AMA prescriptions :

- For tests / investigations at empanelled private hospitals on the recommendation of the AMA — the medical prescription is treated as valid for **single use within a period of 2 weeks (fourteen days)** from the date of prescription, unless otherwise specified in the prescription. The prescription requires revalidation after expiry of the validity period.
- Prescription / advice issued by the AMA for any *procedure* is valid for three months.

 **LEGISLATIVE NOTE:** As per the operative text of the CS(MA) Rules compilation, the AMA prescription for tests / investigations at empanelled hospitals is valid for single use within **2 weeks** from the date of prescription (not 30 days as stated in some older material), requiring revalidation thereafter. For *procedures*, the AMA advice remains valid for three months. Prescriptions for chronic / long-term illnesses (e.g. diabetes, hypertension) are separately valid up to six months.

(9) Stay in hospital — the average stay as in-patient is reckoned as **two weeks**; any stay beyond **three weeks** is considered a prolonged stay.

RULE 3 — MEDICAL ATTENDANCE

(1) A Government servant shall be entitled, free of charge, to medical attendance by the Authorized Medical Attendant.

(2) Where a Government servant is entitled under sub-rule (1), free of charge, to receive medical attendance, any amount paid by him on account of such medical attendance shall, on production of a certificate in writing by the AMA in this behalf, be reimbursed to him by the Central Government.

Note: If the controlling authority rejects the claim under any condition, the claimant may submit an appeal to the appropriate authority within a period of **forty-five days** from the date of receipt of the order rejecting the claim.

GOVERNMENT DECISIONS / CLARIFICATIONS

(a) The first four consultations / visits should be completed within a period of ten days from the date of commencement of treatment in respect of one single and continuous spell of illness for a particular disease.

(b) In Indian Systems of Medicine and Homoeopathy, the 10-day limit may be increased up to **40 days** by the AMA. The total number of consultations during the 40 days shall be four.

(c) A maximum of 15 (fifteen) injections may be administered for non-prolonged treatment at the consulting room of the AMA.

LEGISLATIVE NOTE: The extension limit for treatment under Indian Systems of Medicine (Ayurveda, Unani, Siddha) and Homoeopathy is **40 days** (not 20 days as stated in some older material), during which a total of four consultations are permissible, reckoned from the first date of consultation. This reflects the operative CS(MA) Rules position.

EXAM TIP: If treatment exceeds 4 consultations OR 15 injections within the prescribed period, it becomes '**prolonged treatment**' and the patient must be referred to the OPD of a Government / recognized hospital (see Rule 6 Note (a)). Allopathy limit — 10 days; ISM & Homoeopathy — 40 days; both capped at 4 consultations.

RULE 4 — TRAVELLING ALLOWANCE FOR MEDICAL ATTENDANCE JOURNEYS

(1) When the place at which a patient falls ill is more than **five miles (8 km)** by the shortest route from the consulting room of the AMA —

- (a) the patient shall be entitled to travelling allowance for the journey to and from such consulting room; or
- (b) if the patient is too ill to travel, the AMA shall be entitled to travelling allowance for the journey to and from the place where the patient is.

(2) An application for travelling allowance under sub-rule (1) shall be accompanied by a certificate in writing by the AMA stating that medical attendance was necessary, and (if under clause (b)) that the patient was too ill to travel.

Travelling Allowance — Government Decisions

(1) The attendant will be entitled to travelling allowance both ways at the rate admissible to a member of the family of the Government servant.

(2) Kidney donor:

- (a) Donor is a private person — TA admissible at the rates applicable to the recipient Government servant.
- (b) Donor is another Government servant or his family member — TA admissible at the rates applicable to the donor Government servant.

(3) Special concession to TB and Cancer patients (rail travel concession for admission, discharge, re-examination or periodical check-up):

Sl.	For Whom Available	Nature of Concession (CS(MA) Rule)
1	Patient travelling accompanied by an attendant	Patient and attendant — ½ of normal journey fare

Sl.	For Whom Available	Nature of Concession (CS(MA) Rule)
2	Patient travelling alone	Single journey ticket on $\frac{1}{4}$ of the normal fare

 **LEGISLATIVE NOTE:** The above are the legacy CS(MA) Rule entitlements. Separately, under Indian Railways policy (IRCA Coaching Tariff read with Railway Board circulars), cancer patients get 100% concession in Sleeper / 3AC, 75% in 2S / SL / CC / FC and 50% in 1AC / 2AC; TB patients travelling to / from a sanatorium get 75% in SL / 2S. The suspension of senior-citizen concessions w.e.f. 20.03.2020 (Railway Board CC-22 of 2020) does NOT affect these medical concessions, which continue independently. [Ref: Railway Board CC-22/2020; IRCA Coaching Tariff, Part-I]

RULE 5 — CONSULTATION WITH SPECIALIST

(1) If the AMA is of opinion that the case of a patient is of such a serious or special nature as to require medical attendance by some person other than himself, he may, with the approval of the Chief Administrative Medical Officer of the State (to be obtained beforehand unless the delay entails danger to the health of the patient) —

- (a) send the patient to the nearest specialist or other medical officer; or
- (b) if the patient is too ill to travel, summon such specialist or other medical officer to attend upon the patient.

(2) A patient sent under clause (a) shall, on production of a certificate in writing by the AMA, be entitled to travelling allowance for journeys to and from the headquarters of the specialist.

(3) A specialist or other medical officer summoned under clause (b) shall, on production of a certificate in writing by the AMA, be entitled to travelling allowance for the journey to and from the place where the patient is.

GOVERNMENT DECISIONS / CLARIFICATIONS

(a) Testing of eyesight for glasses at recognized hospitals by specialists — once in every three years on the recommendation of the AMA. The fee paid to the specialist will be reimbursed.

RULE 6 — MEDICAL TREATMENT

(1) A Government servant shall be entitled, free of charge, to treatment —

- (a) in such Government hospital at or near the place where he falls ill as can, in the opinion of the AMA, provide the necessary and suitable treatment; or
- (b) if there is no such hospital as referred to in sub-clause (a), in such hospital other than a Government hospital at or near the place as can, in the opinion of the AMA, provide the necessary and suitable treatment.

(2) Where a Government servant is entitled under sub-rule (1), free of charge, to treatment in a hospital, any amount paid by him on account of such treatment shall, on production of a certificate in writing by the AMA, be reimbursed to him by the Central Government.

Note: Appeal against rejection of a claim — within forty-five days (as in Rule 3).

Notes / Government Decisions

(a) Prolonged Treatment — where treatment exceeds 4 consultations / visits within 10 days from the date of commencement of treatment, OR more than 15 injections, the patient should be referred to the OPD of a Government / recognized hospital.

(b) Treatment for Immunizing and Prophylactic Purposes — reimbursable for treatment of self or family members in the case of communicable diseases only, viz. Cholera, Typhoid group of fevers (TAB), Plague, Diphtheria, Whooping cough, Tetanus and Poliomyelitis.

(c) Vaccination for Hepatitis B is reimbursable.

 **LEGISLATIVE NOTE:** Pursuant to the COVID-19 pandemic and subsequent MoHFW OM's, COVID-19 vaccination, testing (RT-PCR / RAT) and treatment were also made admissible for reimbursement under CS (MA) Rules and CGHS for the periods notified.

RULE 7 — TREATMENT AT RESIDENCE

(1) If the AMA is of opinion that, owing to the absence or remoteness of a suitable hospital or to the severity of the illness, a Government servant cannot be given treatment as provided in clause (a) of sub-rule (1) of Rule 6, the Government servant may receive treatment at his residence.

(2) A Government servant receiving treatment at his residence under sub-rule (1) shall be entitled to receive, towards the cost of such treatment incurred by him, a sum equivalent to the cost of such treatment as he would have been entitled, free of charge, to receive under these Rules if he had not been treated at his residence.

(3) Claims for sums admissible under sub-rule (2) shall be accompanied by a certificate in writing by the AMA stating — (a) his reasons for the opinion referred to in sub-rule (1); and (b) the cost of similar treatment referred to in sub-rule (2).

Note: If the AMA certifies that the Government servant required hospital treatment but no accommodation was available at the recognized hospital, the fees paid for medical treatment at the patient's residence may be reimbursed.

RULE 8 — OTHER MEDICAL FACILITIES

(1) Charges for services rendered in connection with, but not included in, medical attendance on or treatment of a patient entitled, free of charge, to medical attendance or treatment under these Rules, shall be determined by the AMA and paid by the patient.

(2) If any question arises as to whether any service is included in medical attendance or treatment, it shall be referred to the Government and the decision of the Government shall be final.

Notes / Government Decisions on Other Medical Facilities

- (a) Reimbursement of expenditure incurred on treatment of sterility is admissible.
- (b) Expenses incurred in connection with sterilization operation are refundable.
- (c) Expenses incurred by Central Government servants on Medical Termination of Pregnancy are reimbursable, provided the MTP has been performed at Government or other institutions / hospitals approved under the Medical Termination of Pregnancy Act, 1971

 **LEGISLATIVE NOTE:** The MTP (Amendment) Act, 2021 (Act No. 8 of 2021) extended the upper gestation limit for termination from 20 to 24 weeks for special categories of women (rape / incest survivors, minors, women with disabilities, etc.) and eased the requirement of medical opinion for pregnancies up to 20 weeks. Reimbursement now applies to MTPs performed in compliance with the amended Act and the MTP (Amendment) Rules, 2021.

- (d) Reimbursement of complete denture — up to Rs. 2,000/- (complete); Rs. 1,000/- (one jaw).
- (e) Reimbursement of cost of artificial appliances (purchase / repair / replacement). Maintenance cost is to be borne by the beneficiary.
- (f) Re-issue of appliances after — 5 years for adults; 2 years for children (except motorized wheelchair and tricycle).
- (g) Motorized wheelchair and tricycle — re-issued after 5 years.
- (h) Prosthetic components and orthotic joints used in appliances shall have BIS / CE certification for reimbursement and must be fabricated by firms employing qualified prosthetists / orthotists.
- (i) Up to age 12 years is considered the physical growth age.
- (j) Items not in annexures: where item cost is below Rs. 50,000/-, may be purchased on three quotations with permission of the HOD; where cost exceeds Rs. 50,000/-, prior permission of the Additional Director, CGHS of the concerned city / DDG is required, on three quotations and approval of the Technical Standing Committee.
- (k) The list of items and rates is to be revised every 5 years.
- (l) Ceiling for reimbursement of digital hearing aid (one ear only). Replacement after 5 years on the basis of a condemnation certificate from a technical expert / approval of CGHS / Government ENT surgeon:

Type of Hearing Aid	Ceiling (Rs.)
Body worn / Pocket type	3,000/-
Analogue BTE	7,000/-
Digital BTE	8,000/-
Digital ITC / CIC	9,000/-

LEGISLATIVE NOTE: Earlier interim ceilings for Digital BTE (Rs. 15,000) and Digital ITC/CIC (Rs. 20,000) were revised and finally settled at the rates above per MoHFW O.M. No. Z.11011/37/2019-EHS (read with O.M. dated 01.12.2020). Warranty of three years applies; the aid must be recommended per CGHS-approved categories, not by brand name.

(m) Ceiling rates for Intra-Ocular Lenses (IOL)

Type of IOL	Ceiling (Rs.)
Hydrophobic Foldable IOL (Multi-piece / Single-piece Aspheric)	5,000/-
Silicon Foldable IOL	3,600/-
Hydrophilic Acrylic IOL	5,800/-
PMMA IOL (AC / PC)	490/-

LEGISLATIVE NOTE: The earlier IOL categories quoted in older material (Hydrophobic Foldable Rs. 5,800 / Hydrophobic Acrylic Rs. 4,500 / PMMA Rs. 500 / Scleral Fixated Rs. 3,500) have been superseded. The current CGHS / CS(MA) ceiling rates are: **Hydrophobic Foldable Rs. 5,000; Silicon Foldable Rs. 3,600; Hydrophilic Acrylic Rs. 5,800; PMMA Rs. 490.** The cost of the IOL is reimbursable in addition to the package rate for cataract surgery, limited to the ceiling or actual, whichever is lower. LASIK surgery and INTACS rings are not reimbursable. [Ref: MoHFW O.M. F.No. S-1413/2005-06/CGHS/R&H; CGHS O.M. No. 536/2012/R&H/CGHS]

(n) Ceiling for reimbursement of Coronary Stents — current ceiling (w.e.f. 01.04.2026, per NPPA Order S.O. 1587(E)): Drug-Eluting Stents (incl. metallic DES and Bioresorbable Vascular Scaffold / biodegradable stents) **Rs. 39,186.03 per stent**; Bare Metal Stents **Rs. 10,762.15 per stent** — both exclusive of GST. Maximum of two (2) stents per patient or actual cost, whichever is lower.

LEGISLATIVE NOTE: The earlier ceiling of Rs. 60,000/- per stent has been superseded. Stent reimbursement is capped per National Pharmaceutical Pricing Authority (NPPA) notifications

under DPCO 2013, revised annually on the Wholesale Price Index (WPI): DES Rs. 38,267.18 / BMS Rs. 10,509.79 (w.e.f. 01.04.2024); DES Rs. 38,933.14 / BMS Rs. 10,692.69 (w.e.f. 01.04.2025); and DES Rs. 39,186.03 / BMS Rs. 10,762.15 (w.e.f. 01.04.2026). Of the maximum two stents permitted, not more than two may be Drug-Eluting, on the advice of a Government Specialist. As the figures are revised each April, always confirm the latest NPPA / CGHS notification. [Ref: NPPA S.O. 1587(E); CGHS O.M. F.No. Z15025/8/2023/DIR/CGHS]

(o) Ceiling for Bariatric Surgery — Rs. 2,25,000/- or actual, whichever is less.

(p) Ceiling for Cochlear Implant — Rs. 5,35,000/- or actual, whichever is less.

✓ VERIFIED: The base ceilings for Bariatric Surgery (Rs. 2,25,000) and Cochlear Implant (Rs. 5,35,000) remain in force as the reference CGHS ceiling rates. Under the tier-based package-rate revision effective 13.10.2025, the amount actually reimbursed is modulated by hospital accreditation (NABH/NABL standard; Non-NABH minus 15%; super-specialty plus 15%) and city tier, subject to ceiling or actual cost, whichever is lower.

(q) **Entitlement for Indoor Treatment** — based on Basic Pay (7th CPC):

Basic Pay (per month) — 7th CPC	Ward Entitlement
Up to Rs. 36,500	General Ward
Rs. 36,501 to Rs. 50,500	Semi-Private Ward
Above Rs. 50,500	Private Ward

📖 LEGISLATIVE NOTE: Substituted for the earlier slab (Up to Rs. 47,600 / Rs. 47,601–63,100 / Rs. 63,101 & above) vide MoHFW O.M. No. S.11011/11/2016-CGHS(P)/EHS dated 28.10.2022, in partial modification of O.M. dated 09.01.2017; clarified vide O.M. dated 22.11.2022. The same revised slab applies to ward entitlement under the CGHS section below.

(r) Annual Medical Examination for Group 'A' officers of Central Civil Services aged 40 years and above: Men — Rs. 2,000/-; Women — Rs. 2,200/- (or actual, whichever is less).

RULE 9 — COUNTERSIGNATURE OF CERTIFICATES

The Controlling Officer of a patient may require that any certificate required by these Rules to be given by the AMA for travelling allowance purposes shall be countersigned —

- (a) in the case of a certificate given by the Principal Medical Officer of a District — by the Chief Administrative Medical Officer of the State; and
- (b) in the case of a certificate given by any other medical officer — by the Principal Medical Officer of the District.

RULE 10 — TRANSFER TO FOREIGN SERVICE

No Government servant shall be transferred to foreign service unless the foreign employer undertakes to afford to him, so far as may be, privileges not inferior to those which he would have enjoyed under these Rules if he had been employed in the service of the Government of India.

RULE 11 — TREATMENT OUTSIDE INDIA

(1) A Government servant shall be eligible to obtain medical treatment outside India, or to claim reimbursement of the cost of medical treatment obtained inside or outside India, in accordance with the provisions of this rule.

(2) A Government servant desirous of availing of medical treatment outside India may make an application through the Department / Ministry to which he is attached, to the Standing Committee established under this rule, in the form specified.

(3) A Government servant desiring to avail medical treatment outside India for himself or for a member of his family for any treatment specified in the Table below shall, subject to the other provisions of this rule, be eligible:

Table — Treatments Eligible for Consideration Abroad

- (1) Complex / high-risk Cardio-Vascular Surgery cases for treatment at centres with extensive experience;
- (2) Bone Marrow Transplant;
- (3) Complex Medical and Oncological Disorders, such as leukaemia and neoplastic conditions;
- (4) Complex high-risk cases in Micro-Vascular and Neuro Surgery for treatment at centres with extensive experience;
- (5) Treatment of extremely complex ailments (other than the above) which, in the opinion of the Standing Committee, can only be treated abroad and fall in the high-risk category.

(4) The Central Government may from time to time review the list of treatment facilities in the Table and make additions or deletions by notification in the Official Gazette.

(5) The Central Government may, for the purposes of this rule, constitute a Standing Committee consisting of:

- (a) Director-General of Health Services, Ministry of Health, Central Government;
- (b) Director-General of Armed Forces Medical Services;
- (c) Director-General of the Indian Council of Medical Research; and
- (d) Joint Secretary in the Ministry of Health and Family Welfare (Convener).

(6) On receipt of an application, the Standing Committee may, after due consideration and on being satisfied that the ailment / treatment can be treated only outside India, issue a certificate of approval.

(7) The Central Government may authorize reimbursement of expenditure on medical treatment obtained outside India where prior approval could not be obtained due to circumstances beyond control, provided all other conditions are fulfilled.

(8) The Standing Committee may, in the interest of the Government servant or family member, recommend one attendant to accompany; the expenditure incurred is also reimbursable.

(9) Where adequate facility is available within India, the Standing Committee shall record such finding and authorize treatment in such Indian institution; the cost shall be reimbursed.

(10) For the purposes of sub-rule (9), the Ministry of Health, in consultation with the Standing Committee, shall from time to time notify the names of such institutions along with the ailments / treatments available.

(11) The scale of expenditure and eligibility shall be identical to that of an officer of the Indian Foreign Service of corresponding grade in the Ministry of External Affairs under the Assisted Medical Attendance Scheme in force.

GENERAL GOVERNMENT DECISIONS / PROCEDURAL MATTERS

1. Procedural Decisions on Medical Claims

(1) Duplicate cash memos in case of loss must be duly countersigned by the AMA.

(2) Cash memoranda attached to medical claims may be required to be cancelled to prevent dual claims.

(3) The designation and degree of the doctor must be indicated on the claim papers.

(4) Time-limit for medical claims submission :

Category of Claim	Time-limit
Completion of out-door treatment	6 months
Discharge from hospital (in-patient)	6 months
Adjustment of advance after discharge	1 month

 **LEGISLATIVE NOTE:** Substituted for the earlier '3 months' time-limit vide MoHFW O.M. No. S.14025/19/2015-MS/CGHS(P) and subsequent orders standardizing the limit to 6 months for

all out-patient and in-patient claims, applicable to serving employees, pensioners and family pensioners.

(5) Approval limit of medical reimbursement in emergency: there is no financial limit; up to Rs. 2 lakhs — Head of Office (not below the rank of Joint Secretary); above Rs. 2 lakhs — IFA (Integrated Financial Adviser).

2. Grant of Medical Advance

(1) Ordinarily, not more than one advance shall be granted in respect of the same illness or injury.

(2) Advance limit:

- (a) Rs. 10,000/- — for indoor treatment in hospital and out-patient treatment for illnesses such as cancer, where the duration of treatment is 3 months or less.
- (b) 80% of estimated cost or Rs. 36,000/-, whichever is less — for TB where the duration of treatment exceeds 3 months.
- (c) 90% of the package deal (wherever it exists) — for major illnesses such as bypass surgery, kidney transplant, major cancer treatment, etc.

(3) The advance is paid directly to the hospital concerned on receipt of an estimate from the treating Physician / Medical Superintendent of the hospital.

3. Treatment of Special Diseases

(1) **Mental Diseases** — duration of treatment may be extended for a maximum period of three years.

(2) Treatment should be allowed initially for six months, further extendable in blocks of six months, in mental diseases.

 **LEGISLATIVE NOTE:** The Mental Healthcare Act, 2017 (Act No. 10 of 2017), w.e.f. 29.05.2018, replaced the Mental Health Act, 1987. Section 21 guarantees the right to access mental healthcare including services run / funded by Government. Treatment for mental illness under CS(MA) / CGHS is now read with the 2017 Act, the MHC Rules 2018 and the RPwD Act, 2016.

(3) Tuberculosis: the maximum period of treatment from the date of detection, for purposes of reimbursement, shall be restricted to 24 months as certified by the TB specialist / Medical Attendant.

(4) Cancer: treatment can be availed in any recognized cancer institute on the recommendation of the AMA.

(5) Post-treatment check-up suggested is also admissible without the recommendation of the AMA.

(6) Diabetes: the cost of medicines may be reimbursed even for a period beyond three months on the advice of the AMA.

4. Concession for Families

(1) The family of a Government servant is entitled to receive medical attendance and treatment at the hospital at which the Government servant himself is entitled.

(2) **“Family”** for the purposes of these Rules means: wife or husband, parents, widowed sisters, widowed daughters, minor brothers, children, step-children, separated / divorced daughters, and step-mother — wholly dependent upon the Government servant, and normally residing with the Government servant or, in the case of parents, at another station.

(3) Age limit of dependents:

Sl.	Member	Age Limit / Conditions
1	Son / adopted legal child	Until he starts earning OR attains 25 years OR gets married, whichever is earlier.
2	Daughter	Until she starts earning OR gets married — irrespective of age limit, whichever is earlier.
3	Son with permanent disability (physical / mental)	Irrespective of age limit.
4	Dependent divorced / abandoned / separated / widowed daughters, AND dependent unmarried / divorced / abandoned / separated / widowed sisters	Irrespective of age limit.
5	Minor brother(s)	Up to attaining majority.
6	Permanently disabled brother(s)	Irrespective of age limit.

(4) Women's hospitals are admissible for male children up to 6 years of age.

(5) A member of the family is treated as a dependent only if his / her income from all sources (including pension and family pension) is less than Rs. 9,000/- plus the dearness allowance thereon

- (a) The term 'children' includes children adopted legally.
- (b) The term 'wife' includes more than one wife (where lawfully recognized).
- (c) Parents may reside either with the Government servant or with the family members at another station.

(6) When both husband and wife are Government servants, both can claim reimbursement of medical expenses for their respective parents.

(7) Option to include Parents OR Parents-in-law

Earlier this option was available only to female Government employees. The facility to choose either one's own parents or parents-in-law as dependent family members was extended to male CGHS beneficiaries (OM dated 26.07.2023) and thereafter to beneficiaries under CS(MA) Rules, 1944 (OM dated 28.03.2024). A male Government employee may exercise a one-time option only for choosing either his parents or his parents-in-law as dependents.

❖ **LATEST AMENDMENT:** Vide MoHFW O.M. No. S.14025/15/2024-EHS dated 13.05.2026, it is re-iterated that the parents / parents-in-law option applies to all eligible beneficiaries under both CGHS and CS(MA) Rules, 1944, subject to the prescribed dependency conditions. Crucially, the option is **one-time and irreversible** — once exercised in favour of parents, it cannot later be changed to parents-in-law (or vice-versa), including on the demise of the parents or otherwise. [Ref: MoHFW O.M. dated 13.05.2026, read with OM dated 26.07.2023 and 28.03.2024]

(8) The restriction of two children is NOT applicable for claiming medical reimbursement.

(9) A Government employee under suspension is entitled to reimbursement of all medical expenditure at par with working employees.

(10) **“Disability”** — the earlier list of 7 disabilities (under the PwD Act, 1995) has been replaced by the list of 21 disabilities recognized under the Rights of Persons with Disabilities Act, 2016 (effective 19.04.2017):

Group	Specified Disabilities (RPwD Act, 2016)	Examples / Definitions
Physical	1. Locomotor Disability 2. Leprosy Cured Persons 3. Cerebral Palsy 4. Dwarfism 5. Muscular Dystrophy 6. Acid Attack Victims	Loss of mobility; deformity post-leprosy; non-progressive neurological condition; short stature; hereditary muscle disease; disfigurement from acid attack.
Visual	7. Blindness 8. Low Vision	As certified per RPwD Rules, 2017.

Group	Specified Disabilities (RPwD Act, 2016)	Examples / Definitions
Hearing	9. Deaf 10. Hard of Hearing	Hearing loss $\geq$ 70 dB / 60–70 dB.
Speech & Language	11. Speech and Language Disability	Permanent disability arising out of laryngectomy, aphasia, etc.
Intellectual	12. Intellectual Disability (incl. Specific Learning Disability) 13. Autism Spectrum Disorder	Significant limitations in intellectual functioning and adaptive behaviour.
Mental Behaviour	14. Mental Illness	Substantial disorder of thinking, mood or perception.
Chronic Neurological	15. Multiple Sclerosis 16. Parkinson's Disease	Progressive degenerative neurological conditions.
Blood Disorders	17. Haemophilia 18. Thalassaemia 19. Sickle Cell Disease	Hereditary blood disorders.
Multiple	20. Multiple Disabilities incl. Deafblindness 21. Any other category notified by the Central Government	Combination of two or more specified disabilities.

 **LEGISLATIVE NOTE:** The RPwD Act, 2016 (Act No. 49 of 2016) replaced the Persons with Disabilities Act, 1995, coming into force on 19.04.2017. The recognized categories increased from 7 to 21, and reservation in Government employment for persons with benchmark disabilities rose from 3% to 4%. The definition of 'acid attack victim' was further clarified by a Government notification in June 2026.

(11) “Confinement, pre-natal and post-natal treatment” means treatment received by the wife of a Government servant before and after child-birth for physiological or other disability attributable to child-birth.

CENTRAL GOVERNMENT HEALTH SCHEME (CGHS)

CENTRAL GOVERNMENT HEALTH SCHEME (CGHS)

(1) CGHS — Coverage and Cities

Presently approximately 47.44 lakh (~4.74 million) beneficiaries are covered under CGHS in 80 cities across India, with expansion to 100 cities approved in a phased manner. CGHS was established on 1st July, 1954 (originally as the Contributory Health Service Scheme in Delhi) and functions under the Ministry of Health & Family Welfare, with headquarters at CGHS Bhawan, R.K. Puram, New Delhi.

 **LEGISLATIVE NOTE:** Substituted for the older figures of “45 lakh beneficiaries” and “85 cities”. Latest figures per official sources (cghs.mohfw.gov.in): ~47.44 lakh beneficiaries; 80 functional cities; ~2,486 empanelled private hospitals and diagnostic centres, with expansion to 100 cities under implementation.

(2) CGHS provides healthcare through the following systems of medicine:

- (a) Allopathic
- (b) Homoeopathic
- (c) Indian Systems of Medicine — (i) Ayurveda; (ii) Unani; (iii) Siddha; (iv) Yoga

(3) Eligibility:

- (a) All Central Government employees paid from the Central Civil Estimates (except Railways and Delhi Administration), including dependent family members residing in CGHS-covered areas.
- (b) Pensioners of the Central Government (except Railways and Armed Forces) and their families.
- (c) Widows of Central Government pensioners in receipt of family pension.
- (d) Members of Parliament (sitting and former), Judges of the Supreme Court & High Courts, Freedom Fighters, employees of certain autonomous bodies, Railway Board employees, accredited journalists (PIB) and other notified categories.

(4) CGHS Contribution & Ward Entitlement (7th CPC)

For serving Central Government employees residing in CGHS-covered areas, obtaining a CGHS card is compulsory. Monthly contribution (deducted from salary) per the 7th Pay Commission (effective 1st February, 2017):

Level in Pay Matrix (7th CPC)	Contribution per month (Rs.)
Level 1 – 5	250
Level 6	450
Level 7 – 11	650
Level 12 and above	1,000

LEGISLATIVE NOTE: Effective date corrected from “1.1.2017” to “1.2.2017” — per MoHFW O.M. dated 09.01.2017 and modification dated 13.01.2017, the revised CGHS subscription rates took effect from 1st February, 2017.

Pensioners / family pensioners contribute the amount they were subscribing at the time of retirement / death of the Government servant. They may pay yearly, or one-time (10 years = 120 months) for life-long validity of the pensioner CGHS card.

Ward Entitlement in CGHS-Empanelled Private Hospitals — Revised

Basic Pay (per month) — 7th CPC	Ward Entitlement
Up to Rs. 36,500	General Ward
Rs. 36,501 to Rs. 50,500	Semi-Private Ward
Above Rs. 50,500	Private Ward

EXAM TIP: This is the most frequently mis-quoted figure in older material. Remember: General — up to Rs. 36,500 / Semi-Private — Rs. 36,501 to Rs. 50,500 / Private — above Rs. 50,500 (vide O.M. dated 28.10.2022).

(5) CGHS Card Colour Scheme (plastic cards now supplemented by e-CGHS cards):

Beneficiary Category	Card Colour
Serving Government employees	Blue
Pensioners, ex-MPs, Freedom Fighters, etc.	Green
Members of Parliament (sitting)	Red
Autonomous Body beneficiaries / journalists	Yellow

 **LEGISLATIVE NOTE:** CGHS has progressively migrated to digital e-CGHS cards (downloadable via the beneficiary portal) with Aadhaar / ABHA ID linkage. Plastic cards continue to be issued but e-cards are valid for all purposes. The CGHS online system has also been integrated with the 'Bhavishya' portal for retiring employees (MoH&FW O.M., 2026).

(6) A plastic duplicate CGHS card may be obtained against a lost card by applying to the concerned Additional Director along with a copy of the FIR, on payment of Rs. 50/-.

(7) Fixed Medical Allowance (FMA)

FMA is granted to Central Government pensioners / family pensioners residing in areas not covered under CGHS, currently at Rs. 1,000/- per month (w.e.f. 01.07.2017). FMA has evolved from Rs. 100 (1997) → Rs. 300 (2008) → Rs. 500 (2014) → Rs. 1,000 (2017).

 **LEGISLATIVE NOTE:** (i) Vide DoP&PW O.M. dated 07.02.2025, FMA of Rs. 1,000/- per month was formally extended to Central Government employees retired under the National Pension System (NPS), released quarterly through the bank subject to a life certificate. (ii) A proposal to enhance FMA from Rs. 1,000 to Rs. 3,000 per month (per Parliamentary Committee / DRPSC recommendations) has been placed before the **8th Central Pay Commission** and was an agenda item at the 49th NC (JCM) meeting, but has NOT yet been implemented as on July 2026.

(8) A serving employee can apply for a pensioner CGHS card along with pension papers; the pensioner card is issued on the day of retirement (if applied for at least 6 weeks before retirement) and activated from the next day.

(9) Where both husband and wife are Central Government employees, they are NOT eligible for separate CGHS cards. The spouse drawing higher pay makes the card; the other spouse avails CGHS benefits as a member.

(10) The service card is valid till the date of retirement, if the employee is otherwise eligible.

(11) On transfer of a serving employee to a non-CGHS-covered area, the service card remains valid for family members up to 6 months after transfer, provided CGHS contribution for 6 months is paid in advance.

(12) On transfer of a serving CGHS beneficiary from one department / ministry to another in the same city, the same CGHS card continues; the new department shall inform CGHS for database updation.

(13) Supply of Medicines from Wellness Centres:

- (a) Medicines for chronic diseases may be issued for 3 months at a time by any CGHS doctor against a valid specialist prescription.
- (b) If the beneficiary is going abroad, medicines may be issued up to 6 months at a time with permission of the Additional Director (with valid prescription, CGHS card and proof of travel).

(14) Hospitalization in Emergency:

- (a) In emergency, a CGHS beneficiary may be admitted to any Government / private hospital; reimbursement is at CGHS rates. Full reimbursement may be allowed in specific cases on the recommendation of the Technical Standing Committee (OM dated 20.02.2009).
- (b) The private hospital must issue an emergency certificate stating in detail the condition that justified emergency.
- (c) CGHS-empanelled hospitals CANNOT refuse admission in emergency and must provide treatment on credit to pensioner beneficiaries and other entitled categories (even if from a different CGHS city).

(15) For treatment in emergency at a non-empanelled hospital, OR expenditure incurred at an empanelled hospital, a Medical Reimbursement Claim (MRC) must be submitted by the beneficiary.

(16) MRC submission time-limit — the claim must be submitted to the concerned department by serving employees and by pensioner beneficiaries within 6 months of discharge from hospital.

 **LEGISLATIVE NOTE:** Substituted for the earlier '3 months' limit vide MoHFW O.M. No. S.14025/19/2015-MS/CGHS(P). The limit is standardized to 6 months for all medical claims (out-patient, in-patient and follow-up).

(17) Empanelled Hospitals & Diagnostic Centres

Approximately 2,486 private hospitals and diagnostic centres are empanelled under CGHS as on 2025-26. All existing Memoranda of Agreement (MoAs) ceased to be valid w.e.f. 13.10.2025; hospitals were required to obtain fresh empanelment under the revised package rates (validity of the transitional empanelment was extended up to 15.08.2026 for the process under HEM 2.0).

 **LEGISLATIVE NOTE:** Substituted for the older figures “1,303 empanelled hospitals and 206 diagnostic centres as on 15.06.2020”. The network expanded materially under the 2025 reforms, with a large number of hospitals freshly empanelled through the Hospital Engagement Module (HEM 2.0).

(18) Monetary ceiling for free diet for CGHS beneficiaries — pay / pension / family pension of Rs. 44,900/- per month.

(19) Monetary ceiling for free diet for beneficiaries suffering from TB or mental disease — pay / pension / family pension of Rs. 69,700/- per month.

(20) For any pensioner / family pensioner entitled to CGHS, the rate of contribution is determined with reference to the level of pay he / she would have drawn in the post held at the time of retirement / death, had he / she continued in service.

5. Major CGHS Reforms — Effective 13.10.2025

— the MoHFW announced a comprehensive overhaul of CGHS package rates and the empanelment framework, effective 13.10.2025 — the first major rate revision since 2014.

(a) Tier-Based City Classification

CGHS cities are now classified into Tier I, Tier II and Tier III for differential package-rate purposes. North-Eastern States and the UTs of Jammu & Kashmir and Ladakh follow Tier II rates.

(b) Accreditation-Based Differential Rates

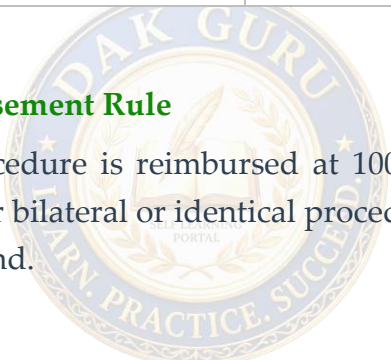
Hospital Category	Rate Differential
NABH / NABL accredited hospital	Standard CGHS rates
Non-NABH / Non-NABL hospital	15% reduction from standard rates
Super-specialty hospital	15% premium over accredited rates

(c) Multiple Procedure Reimbursement Rule

In a single session: the first procedure is reimbursed at 100%, the second at 50%, and each subsequent procedure at 25%. For bilateral or identical procedures on different sites — full rate for one side and 50% for the second.

(d) Re-Empanelment Mandate

All existing MoAs with CGHS-empanelled private hospitals stood terminated at 12:00 AM on 13.10.2025. Hospitals had to accept the new MoA terms and execute fresh MoAs through the Hospital Engagement Module on the CGHS portal, failing which they were to be de-panelled.

 **LEGISLATIVE NOTE:** Cashless treatment is now also available at AIIMS New Delhi, PGIMER Chandigarh and JIPMER Puducherry. Rates for nearly 2,000 procedures were revised — the first comprehensive overhaul in over a decade. Refer to Annexure-I of cghs.mohfw.gov.in for the full revised rate list for the relevant city tier.

6. Recent Pay Commission & DA-Linked Updates

— developments affecting CGHS subscription and FMA quantum (indirect impact via pay slabs):

- Dearness Allowance / Relief for Central Government employees and pensioners raised to 60% w.e.f. 01.01.2026; DA / DR confirmed at 63% w.e.f. 01.07.2026.

- The 8th Central Pay Commission has been constituted; its Terms of Reference include re-examination of CGHS subscription and the FMA quantum (proposal to raise FMA to Rs. 3,000).
- The 8th CPC is receiving memoranda from the JCM (Staff Side) and pensioner associations, with regional interactions ongoing through 2026.



QUICK REVISION CAPSULE FOR LDCE-IP / PS GROUP B

KEY POINTS — RAPID REVISION

Item / Provision	Quick Reference
Notification of CS(MA) Rules	1st January, 1944
Constitutional basis (post-1950)	Article 313 (continuance in force)
AMA consultation fee — PG / Specialist	Rs. 350 (first & subsequent), w.e.f. 20.09.2023
AMA consultation fee — Graduate (MBBS)	Rs. 200 (first & subsequent)
AMA injection fee	IM / SC Rs. 25; IV Rs. 50 (no day/night distinction)
Distance threshold for TA	More than 5 miles / 8 km from AMA's consulting room
Allopathy treatment limit	10 days, max 4 consultations
ISM & Homoeopathy treatment limit	40 days, max 4 consultations
Maximum injections (non-prolonged)	15
Average hospital stay (in-patient)	2 weeks; beyond 3 weeks = prolonged
Appeal time-limit (claim rejection)	45 days
MRC submission time-limit	6 months (was 3 months)
AMA prescription — single-use validity (tests)	2 weeks (revalidation thereafter)
AMA prescription — procedure validity	3 months
IVF age / cycles / cap	21–39 years / 3 cycles / Rs. 65,000 per cycle
Eyesight test (glasses)	Once in 3 years
IOL ceilings	Hydrophobic Foldable 5,000 / Silicon 3,600 / Hydrophilic 5,800 / PMMA 490
Coronary stents	Max 2; DES Rs. 39,186 / BMS Rs. 10,762 (NPPA, w.e.f. 1.4.2026)
Bariatric surgery ceiling	Rs. 2,25,000/-
Cochlear implant ceiling	Rs. 5,35,000/-

Item / Provision	Quick Reference
Ward entitlement (7th CPC basic pay)	≤ 36,500 General / 36,501–50,500 Semi-Pvt / >50,500 Private
Special Nurse / Attendant fee	Rs. 150 / Rs. 75 per 12-hour shift
FMA (current)	Rs. 1,000 per month (extended to NPS pensioners)
Free-diet income ceiling (general / TB-mental)	Rs. 44,900 / Rs. 69,700 per month
CGHS cities / beneficiaries	80 cities (→ 100) / ~47.44 lakh
RPwD Act, 2016 — disabilities	21 (was 7 under PwD Act, 1995); 4% reservation
Standing Committee (Treatment Outside India)	DGHS + DG-AFMS + DG-ICMR + JS-MoHFW (Convener)

Key Office Memoranda / Notifications to Remember

Sl.	Reference / Date	Subject
1	MoHFW O.M. No. S.11025/18/2023-EHS dated 20.09.2023	Revised AMA consultation / injection fees (PG 350, Graduate 200)
2	MoHFW O.M. dated 09.01.2017 (w.e.f. 01.02.2017)	CGHS subscription per 7th CPC pay-matrix levels
3	MoHFW O.M. No. S.11011/11/2016-CGHS(P)/EHS dated 28.10.2022	Revised ward entitlement (36,500 / 50,500 slabs)
4	MoHFW O.M. No. S.14025/15/2024-EHS dated 13.05.2026	Parents / parents-in-law one-time option (CGHS & CS(MA))
5	DoP&PW O.M. dated 07.02.2025	FMA extended to NPS pensioners
6	MoHFW CGHS Rate Revision (13.10.2025)	Tier-based differential package rates
7	NPPA Order S.O. 1587(E) (w.e.f. 01.04.2026)	Coronary stent ceilings (DES 39,186 / BMS 10,762)
8	MoHFW O.M. F.No. S-1413/2005-06/CGHS/R&H	Revised IOL ceiling rates
9	RPwD Act, 2016 (No. 49 of 2016)	21 disabilities; 4% reservation
10	Mental Healthcare Act, 2017 / MTP (Amdt.) Act, 2021	Replaced 1987 Act / gestation extended to 24 weeks

🎯 **EXAM TIP:** Common LDCE-IP question patterns: (i) match the O.M. with its subject; (ii) state the revised fee / slab / ceiling; (iii) distinguish PwD Act 1995 from RPwD Act 2016; (iv) identify categories NOT covered by CS(MA) Rules; (v) the steps for emergency hospitalization at a non-empanelled hospital; (vi) the AMA fee and treatment-day limits.

Final Note for Aspirants

This booklet is a study aid only. For LDCE-IP, PS Group B and other departmental examinations, candidates should also refer to the original Office Memoranda on cghs.mohfw.gov.in, the Postal Manual Volume V (where applicable) and the latest Compendium of Establishment Orders. Whenever in doubt, the language of the latest O.M. / Gazette notification prevails.

== End of CS (Medical Attendance) Rules, 1944 — Revised & Updated Edition ==



Prepared exclusively for Dak Guru — A Smart Self-Learning Portal

www.dakguru.com

